

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Salbutamol 100mcg MDI and Amoxicillin 500mg capsules for the treatment of COPD Management

Patient Group Direction, version 002, issued 11 September 2026. Supersedes Version 001, 1 November 2025.

Summary of NICE / NICE CKS Guidelines for COPD Management

COPD Overview

- **Definition:** Chronic Obstructive Pulmonary Disease is a progressive condition characterised by persistent airflow limitation due to emphysema and/or chronic bronchitis.
- **Diagnosis:** Confirmed by spirometry showing FEV1/FVC <0.70; GOLD classification (GOLD 1-4) based on FEV1.
- **Aetiology:** Predominantly smoking-related; occupational/environmental exposures possible.

GOLD Classification and Exacerbations

- **GOLD 1:** FEV1 ≥80% predicted (mild); low exacerbation risk.
- **GOLD 2:** FEV1 50-79% predicted (moderate); higher exacerbation risk.
- **GOLD 3-4:** FEV1 <50% predicted (severe/very severe); high exacerbation risk.
- **Exacerbation:** Acute worsening with increased dyspnoea, cough, and change in sputum characteristics (colour, volume).

Exacerbation Assessment

- **Infective exacerbation:** Purulent sputum (yellow/green) indicates bacterial involvement.
- **Symptoms:** Increased dyspnoea, cough, sputum production, haemoptysis, fever.
- **Severity:** Assess SpO2, RR, accessory muscle use; consider hospital referral if severe (SpO2 <88%, RR >25, inability to speak).

Pharmacological Management

- **Rescue therapy:** Short-acting beta-2 agonists for acute symptom relief (salbutamol).
- **Infective exacerbation:** Antibiotics for purulent sputum (amoxicillin for lower respiratory tract infection).
- **Oxygen therapy:** Target SpO2 88-92%; managed separately from this PGD.

Patient Group Direction

for the supply/administration of

Salbutamol 100mcg MDI inhaler
for the treatment of
COPD Management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Acute symptom relief in adults with COPD presenting with acute exacerbation or breathlessness
Inclusion criteria	Adults aged 18 years and over Confirmed diagnosis of COPD (documented spirometry and GOLD classification) Acute exacerbation or episodes of breathlessness requiring symptom relief Capable of using inhaler device or willing to use spacer Capable of giving valid informed consent
Exclusion criteria	Known hypersensitivity to salbutamol or other beta-2 agonists Severe exacerbation with hypoxia (SpO₂ <88%) requiring emergency referral and oxygen therapy Acute distress with inability to speak, cyanosis, or signs of respiratory

	failure
Cautions	Cardiovascular disease: salbutamol can increase heart rate and blood pressure; assess cardiac risk Hypertension: monitor blood pressure; beta-2 agonists may worsen Coronary artery disease or recent MI: assess risk/benefit; monitor for angina Diabetes mellitus: monitor blood glucose (hyperglycaemia possible) Hyperthyroidism: beta-2 agonists can worsen symptoms Hypokalaemia: may be worsened; monitor K⁺ levels Pregnancy: generally safe; ensure informed consent
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Salbutamol 100mcg metered-dose inhaler (MDI)
Legal category	POM
Storage	Store below 25°C. Keep away from heat and direct sunlight. Shake well before each use.
Route/method of administration	Inhalation via MDI, with or without spacer
Dose and frequency	1 to 2 puffs (100 to 200 micrograms) as required for breathlessness, up to 4 times in 24 hours Use for acute symptom relief during exacerbations or ongoing dyspnoea Use spacer device to optimise drug delivery if patient is not experienced with MDI Maximum 8 puffs in 24 hours under this PGD. A need for more than this, for relief more often than every 4 hours, or for the reliever on most days, is uncontrolled or exacerbating disease: same-day GP or urgent care referral. Ten puffs through a spacer with no relief is an emergency: call 999.
Quantity to be administered and/or supplied	1 MDI inhaler (200 doses)
Maximum or minimum treatment period	One inhaler (200 actuations) per supply. Maximum 2 supplies in any 12 months under this PGD; a third request is a GP review of the patient's COPD, not a further supply.
Adverse effects	Common: fine tremor of hands, headache, nervousness, tachycardia,

	palpitations Uncommon: muscle cramps, anxiety, dizziness Rare: hypersensitivity reactions, arrhythmias, myocardial ischaemia, angina
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with each medication.
Follow-up advice to be given to patient or carer	Use the rescue inhaler (salbutamol) as needed when you experience breathlessness. Use a spacer device if you have difficulty coordinating MDI actuation. Complete the full course of amoxicillin even if symptoms improve. Take amoxicillin at regular intervals, ideally 1 hour before or 2 hours after meals for best absorption. If using oral contraception, use additional contraceptive methods during and for 7 days after the antibiotic course. Monitor your sputum colour: purulent (yellow/green) sputum suggests continued infection. Seek immediate medical attention if symptoms worsen despite treatment, or if you develop fever, persistent chest pain, or

Valid from date	Expiry date		
11/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction

for the supply/administration of

Amoxicillin 500mg capsules

for the treatment of

COPD Management

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of infective acute exacerbations of COPD characterised by purulent sputum (bacterial lower respiratory tract infection)
Inclusion criteria	Adults aged 18 years and over Confirmed diagnosis of COPD with spirometry evidence Acute exacerbation with purulent sputum (yellow/green coloration indicating bacterial infection) Able to take oral medication Capable of giving valid informed consent
Exclusion criteria	Known penicillin or beta-lactam allergy Infectious mononucleosis (amoxicillin can precipitate severe rash)

	Severe renal impairment (eGFR <30 mL/min/1.73m²); dose adjustment needed Antibiotic resistance suspected in local resistance patterns - check local microbiology guidance
Cautions	Mild to moderate renal impairment: monitor renal function; dose adjustment may be needed Hepatic impairment: generally safe but monitor liver function Pregnancy and breastfeeding: amoxicillin is generally safe; ensure informed consent Drug interactions: may reduce efficacy of oral contraceptives; advise additional contraception Check local antibiotic resistance patterns before prescribing (Haemophilus influenzae, Moraxella catarrhalis, Streptococcus pneumoniae coverage) Use wider-spectrum antibiotics if resistance is a concern in the area
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Amoxicillin 500mg capsules
Legal category	POM
Storage	Store below 25°C. Keep in original container. Protect from moisture.
Route/method of administration	Oral
Dose and frequency	500mg (one capsule) three times daily To be taken on an empty stomach (1 hour before or 2 hours after meals) for optimal absorption Or may be taken with food if GI upset occurs
Quantity to be administered and/or supplied	15 capsules (5-day course)
Maximum or minimum treatment period	5 days
Adverse effects	Very common: diarrhoea, nausea Common: rash (non-allergic), vomiting, abdominal pain, vaginitis Uncommon: indigestion, headache Rare: hypersensitivity reactions (urticaria, Stevens-Johnson

	syndrome, TEN), hepatitis, C. difficile infection, angioedema, anaphylaxis
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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11/9/26	31/7/27		

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Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Version and change record

Version: v002

Issued: 11 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Salbutamol dose, maximum dose and maximum period stated: 1 to 2 puffs up to 4 times in 24 hours, maximum 8 puffs in 24 hours, one inhaler per supply and at most 2 supplies in 12 months, with the same-day and 999 thresholds. The previous version said '2-4 puffs as required', 'may repeat at intervals as needed' and 'ongoing', which is no maximum at all.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Signed on behalf of Get Real Health

Version v002, 11 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		11 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		11 September 2026

Both authorising signatories reviewed this version together on 11 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.